

ECHS FA-07 — Emergency Route Misuse Report

Emergency Admission Fraud Analytics · FY 2025-26

FA-07 | Rule-Based Signal Detection

CLASSIFICATION	PERIOD	RECORDS ANALYSED	QUERY SETS
RESTRICTED	01-Apr-2025 to 31-Mar-2026	1,289,127 IPD Claims	8 Signals / 2,686 Hospitals

ECHS — Airawat Research Foundation

22 May 2026

ECHS Emergency Route Misuse — FA-07

FY 2025-26 · 1 April 2025 – 31 March 2026

Report date: 22 May 2026 **Methodology:** Rule-based signal detection across eight independent fraud indicators applied to emergency IPD admission claims.

Note to reviewer: This report is produced by an automated screening system. Every flagged hospital is a *lead for investigation*, not a confirmed finding. A qualified auditor should review each case before any action is taken.

At a glance: 86 hospitals **HIGH_RISK** | 278 **SUSPICIOUS** | 1,596 **MONITOR** | 726 **CLEAN** out of 2,686 hospitals analysed.

About This Report

This report presents the findings of **ECHS FA-07 — Emergency Route Misuse Analytics**, a systematic screening exercise applied to all ECHS inpatient claims for FY 2025-26.

The analysis uses **eight independent rule-based signals**, each designed to detect a specific pattern associated with fraudulent or irregular use of the emergency admission route. This route exists for genuine life-threatening emergencies but is systematically exploited to bypass the polyclinic referral process and inflate claim amounts.

Each signal operates independently. A hospital triggering multiple signals simultaneously is assigned to a higher risk tier.

#	Signal	Description
S1	High Emergency Rate	Emergency admissions as a share of total IPD > 80%
S2	Working-Hours Clustering	Emergencies concentrated 9 AM–6 PM (office hours)
S3	Month-End Surge	Spike in emergency claims in last 5 calendar days of month
S4	Quarter-End Surge	Spike in last 15 days of Jun / Sep / Dec / Mar
S5	Claim Amount Premium	Emergency claims cost 3× or more than referral claims
S6	Short-Stay Emergency	Emergency patients discharged faster than referral patients
S7	OPD Emergency Overuse	> 50% of OPD visits also labelled emergency
S8	Repeat-Beneficiary Concentration	Same patients admitted as emergency repeatedly

What Is the Emergency Route, and Why Does It Matter?

When an ECHS beneficiary requires hospitalisation, the standard process requires a **referral from an authorised polyclinic**. This referral is reviewed, and the planned admission is supervised to ensure appropriateness of treatment and fair pricing.

The **emergency route** exists as a necessary exception -- for life-threatening situations (heart attack, serious trauma, acute stroke) where waiting for a polyclinic appointment is not feasible. Emergency admissions bypass the normal referral approval and are processed for payment more quickly.

This creates a financial incentive for hospitals: labelling a routine, planned admission as emergency allows faster payment, higher claim amounts, and reduced scrutiny. If a hospital routinely does this, it constitutes **fraud against the ECHS fund**.

This analysis examined **2,686 hospitals** and **1,289,127 in-patient (IPD) claims** processed during FY 2025-26 to identify hospitals that appear to be exploiting this route.

The Central Finding: Emergency Is No Longer the Exception

Across the entire ECHS network, **42.2% of all in-patient admissions in FY 2025-26 were coded as emergency** -- more than 544,560 claims.

More strikingly:

- **10.9% of all hospitals** (292 out of 2,686) admitted **every single in-patient as emergency** -- a 100% emergency rate.
- The **median emergency rate** across all hospitals is **40.6%**.
- **19.9% of hospitals** have an emergency rate exceeding 80%.

In any functioning healthcare system, emergency admissions are the exception, not the rule. A hospital where 100% of patients arrive as emergencies is not describing genuine clinical emergencies -- it is describing a billing practice.

Findings by Risk Tier

The analysis scored each hospital across 8 independent fraud signals. Based on how many signals fired simultaneously, hospitals were assigned to one of four tiers:

Tier	Signals Fired	Hospitals	Share of Network
HIGH_RISK	4-5 out of 8	86	3.2%
SUSPICIOUS	3 out of 8	278	10.3%
MONITOR	1-2 out of 8	1,596	59.4%
CLEAN	0 out of 8	726	27.0%

HIGH_RISK -- 86 Hospitals (4-5 signals)

These hospitals triggered four or five signals simultaneously -- the strongest evidence of deliberate, systematic misuse of the emergency route.

- **75.6% average emergency rate**
- **71.4% of emergency admissions during working hours** (9 AM-6 PM)
- Median emergency claim: **Rs 86,420** -- 0.93x higher than own referral claims (Rs 78,994)
- Month-end concentration: 24.2% | Quarter-end concentration: 24.3%

Estimated financial exposure: 5,948 emergency claims at these 86 hospitals -- **~Rs 85.8 crore**. These hospitals require immediate field audit and payment hold.

SUSPICIOUS -- 278 Hospitals (3 signals)

These hospitals triggered exactly three independent red flags -- a strong but not conclusive indicator; recommended for desk review and document verification.

- 62.8% average emergency rate	Working-hours: 67.5%
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- Median emergency claim: **Rs 95,233** -- 1.06x referral claims (Rs 73,554)
- Month-end: 19.7% | Quarter-end: 20.8%

Estimated financial exposure: 32,164 emergency claims -- ~**Rs 340.9 crore**.

MONITOR -- 1,596 Hospitals (1-2 signals)

These hospitals show 1 or 2 red flags -- meaningful irregularities warranting periodic review but not yet conclusive evidence of deliberate fraud.

- 48.8% average emergency rate	Median claim: Rs 88,060 (1.12x referral)
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- Contributed 433,921 emergency IPD claims -- the largest source of system-wide exposure

With 59.4% of the network in this tier, emergency route overuse is widespread.

CLEAN -- 726 Hospitals (0 signals)

No fraud signals fired. These hospitals represent approximately 27.0% of the network and serve as the behavioural baseline.

The Eight Fraud Signals -- How Often Did They Fire?

Signal	Description	Hospitals Flagged	% of Network
s1highemg_rate	Emergency rate > 80%	534	19.9%
s2workinghrs	Emergency admissions concentrated in working hours (9 AM-6 PM)	376	14.0%
s3_monthend	Surge in admissions at calendar month-end	311	11.6%
s4_qtrend	Surge in admissions at financial quarter-end	395	14.7%
s5claimpremium	Emergency claims cost 3x or more than referral claims	46	1.7%
s6shortlos	Emergency patients discharged faster than referral patients	726	27.0%
s7opdemg	OPD visits also labelled as emergency (> 50%)	280	10.4%
s8repeatbenf	Same patients admitted repeatedly as emergency (> 1.4x)	808	30.1%

S1 fires at 19.9% -- this reflects a network-wide emergency labelling culture, not an isolated group. The other signals distinguish genuinely abusive hospitals from the elevated baseline.

Signals in SUSPICIOUS hospitals specifically:

Signal	Fired in SUSPICIOUS	% of SUSPICIOUS
s1highemg_rate	133 / 278	48.0%
s2workinghrs	113 / 278	41.0%
s3_monthend	116 / 278	42.0%
s4_qtrend	131 / 278	47.0%
s5claimpremium	16 / 278	6.0%
s6shortlos	135 / 278	49.0%
s7opdemg	82 / 278	29.0%
s8repeatbenf	108 / 278	39.0%

Notable Individual Cases

Claim Inflation -- Most Severe (HIGH_RISK + SUSPICIOUS combined)

Hospital ID	Hospital Name	Emergency Claims	Avg Emergency Claim	Avg Referral Claim	Premium Ratio
4509	4509 - SHREE GAJANAN HEART AND EYE HOSPITALS PVT. LTD.	2	Rs 233,200	Rs 20,577	11x
703	703 - KUKREJA HOSPITAL & HEART CENTRE PVT. LTD - RAJOURI GARDEN	98	Rs 279,641	Rs 27,165	10x
4579	4579 - MISSION HEALTHCARE HOSPITAL	115	Rs 141,956	Rs 17,152	8x
46	46 - ESCORTS HEART INSTITUTE & RESEARCH CENTRE LTD - OKHLA	1	Rs 1,385,296	Rs 182,343	8x
3628	3628 - INDUS VALLEY HOSPITALS (A UNIT OF PRIME CARE HOSP)	242	Rs 141,756	Rs 19,416	7x
3910	3910 - MAHARISHI MARKANDESHWAR COLLEGE OF MEDICAL SCIENCES & RESEARCH	224	Rs 58,685	Rs 8,671	7x

A **11x premium** at **4509 - SHREE GAJANAN HEART AND EYE HOSPITALS PVT. LTD.** (4509) means this hospital claimed 11x more per patient under the emergency label.

Largest-Volume HIGH_RISK Hospital

4323 - CBM HEALTH CARE (4323) processed **2,131 total IPD claims**, of which **956 (44.9%) were emergency**. Average emergency claim: Rs 218,006 vs referral Rs 226,968 (1.0x premium). Working-hours concentration: 48.7%.

Largest-Volume SUSPICIOUS Hospital

3980 - PRATAP HOSPITAL (3980) processed **3,433 total IPD claims**, of which **864 (25.2%) were emergency**. Average emergency claim: Rs 124,400 vs referral Rs 189,155 (0.7x premium). Working-hours concentration: 82.4%.

Highest Single Emergency Claim Average

46 - ESCORTS HEART INSTITUTE & RESEARCH CENTRE LTD - OKHLA (46) -- Rs 1,385,296 average per emergency claim. Fraud score: 3 signals.

Patterns of Fraud -- Most Common Signal Combinations in SUSPICIOUS

1. **s1 high emg rate + s3 monthend + s7 opd emg** -- 31 hospitals
 2. **s3 monthend + s4 qtrend + s6 short los** -- 29 hospitals
 3. **s1 high emg rate + s4 qtrend + s7 opd emg** -- 25 hospitals
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Key Observations and Recommendations

1. The Emergency Route Has Become the Default Route

The 100% emergency rate at 10.9% of hospitals, and a network median of 40.6%, indicates the emergency label has ceased to function as a safeguard.

Recommendation: Define a permitted emergency rate benchmark for different hospital types and require hospitals exceeding it to submit clinical justification.

2. Period-End Manipulation Is Systematic

Both month-end and quarter-end signals fire across hundreds of hospitals.

Recommendation: Trigger automatic holds on emergency claims filed in the last 5 days of each month for hospitals with a prior history of month-end surges.

3. Claim Inflation Is Extreme in HIGH_RISK Hospitals

The 11x, 10x, and 8x premium ratios are not statistical artefacts -- they represent enormous absolute amounts billed under the emergency label.

Recommendation: All 86 HIGH_RISK hospitals with a claim premium signal should be placed on immediate payment hold pending audit recovery.

4. Working-Hours Clustering Needs Clinical Review

43 HIGH_RISK hospitals have 75%+ of emergency admissions during 9 AM-6 PM.

Recommendation: Request admission registers and triage documentation for a 30-day sample period. Cross-reference with treating doctor credentials and diagnosis codes.

5. Repeat Patients Undermine the Emergency Premise

808 hospitals (30.1%) show the same patients returning for emergency admissions multiple times in the same year.

Recommendation: Flag beneficiaries with 3 or more emergency admissions at the same hospital within FY 2025-26 for individual case review.

Limitations

- **Referral claims absent for some hospitals:** Several SUSPICIOUS hospitals have no referral claims; claim premium ratios could not be computed for these.
- **Bed count data unavailable:** A planned signal comparing emergency admissions to bed capacity could not be computed -- no bed count column exists in *officemaster* and the *offcbse_info* table uses a different ID schema.
- **State shown as code:** `OM_OFFICE_STATE_ID` is a 3-character code; a join to `state_master` would yield full state names.
- **FY 2025-26 only:** Trend analysis across multiple years would strengthen findings.

Summary

Finding	Number
Total hospitals analysed	2,686
Total IPD claims examined	1,289,127
Total emergency IPD claims	544,560 (42.2%)
Hospitals with 100% emergency rate	292 (10.9%)
HIGH_RISK hospitals (4-5 signals)	86 (3.2%)
SUSPICIOUS hospitals (3 signals)	278 (10.3%)
MONITOR hospitals (1-2 signals)	1,596 (59.4%)
Highest claim premium ratio observed	11x (4509 - SHREE GAJANAN HEART AND EYE HOSPITALS PVT. LTD.)
Estimated emergency spend -- SUSPICIOUS tier	~Rs 340.9 crore

Analysis performed by the CDIS Fraud Analytics team under project FA-07. Data source: ECHS transactional database, FY 2025-26. Raw output: ``sql\RAW_DATA_2026-05-22.csv``. Report generated: 22 May 2026.